OPERATION: Robotic-assisted laparoscopic radical prostatectomy (RALRP) with bilateral pelvic lymph node dissection.

INDICATION: A 62-year-old male with clinically localized, intermediate-risk prostate adenocarcinoma (Gleason score 3+4=7, PSA 7.4 ng/mL, clinical stage cT1c).

PROCEDURE DESCRIPTION: General endotracheal anesthesia was induced, pneumatic compression boots applied, and the patient positioned in a steep 30-degree Trendelenburg position. A six-port transperitoneal robotic access configuration was established using the da Vinci Xi surgical system.

The peritoneum was incised lateral to the umbilical ligaments to enter the space of Retzius. The anterior surface of the prostate and endopelvic fascia were exposed. The endopelvic fascia was incised bilaterally from the apex to the base of the prostate. The puboprostatic ligaments were preserved. The dorsal venous complex (DVC) was selectively controlled and ligated using a 2-0 barbed suture. The bladder neck was transected circumferentially under direct visualization, identifying and preserving the ureteral orifices.

The vasa deferentia were clipped and divided, and the seminal vesicles dissected bilaterally. Denonvilliers' fascia was incised, developing the plane between the posterior prostate and anterior rectum. Bilateral nerve-sparing dissection was performed utilizing an athermal interfascial technique along the neurovascular bundles. The prostatic pedicles were controlled with Weck Hem-o-lok polymer clips. The apical dissection was completed with sharp division of the urethra 2 mm distal to the prostate apex.

Bilateral pelvic lymph node dissection was executed, removing nodal tissue within the obturator fossa and along the external iliac vein. A vesicourethral anastomosis was reconstructed using the Van Velthoven continuous suturing technique with a double-ended 3-0 V-Loc absorbable barbed suture over an 18-French Foley catheter. A saline bladder leak test with 180 mL sterile water demonstrated a watertight, tension-free anastomosis without extravasation. The prostate specimen was removed in an Endocatch bag, ports were closed, and blood loss was under 150 mL.